



GUIDELINE

Engaging with Peer Workers

Scope (Staff):	All staff
Scope (Area):	CAMHS-Wide

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

Aim

To inform and guide all CAMHS staff on the scope of practice for Peer Workers to enable successful engagement and integration as part of the multidisciplinary team.

Risk

- Failure to provide all CAMHS staff with clarity on the roles and responsibilities of Peer Workers will result in inconsistent work practices, and services that do not fully integrate Peer Workers into multidisciplinary teams.
- A lack of role clarity can impact job satisfaction, effectiveness and contribute to psychosocial risk for employees in Peer Work roles.
- Provision of services that do not fully leverage the capacity of Peer Workers to support engagement and the recovery needs of children and young people accessing CAMHS services, and their carers/ family/ significant other.

Background

CAMHS is committed to proactively growing its Lived Experience (Peer) workforce. Peer Workers will be integrated into the multi-disciplinary team (MDT) within new and existing CAMHS services, over time contingent on funding.

Peer Workers are specialised, non-clinical employees¹ that complement the skills and expertise of other mental health professionals, providing a unique perspective based on direct/indirect experience (lived experience) in mental health challenges and recovery.

¹ The National Mental Health Workforce Development Guideline acknowledges and promotes Lived Experience (Peer) Work as a discipline in its own right.

This guideline aims to provide information on how to effectively engage Peer Workers in the admission, assessment, planning and discharge process. For more information on the specific role the job description should be reviewed.

The Lived Experience (Peer) Workforce aligns with a wide variety of guiding documents including the following:

- [Accreditation Requirements for Mental Health Services](#)
- [Mental Health Act 2014 \(WA\)](#)
- [Final Report: Ministerial Taskforce into Public Mental Health Services for Infants, Children, and Adolescents \(ICA\)](#)
- [WA State Priorities Mental Health, Alcohol and Other Drugs 2020 – 2024](#)
- [WA Lived Experience \(Peer\) Workforces Framework](#)
- [Western Australian Mental Health, Alcohol and Other Drug Services Plan 2015–2025.](#)

Key points

- Peer Work in Mental Healthcare is a unique discipline that collaborates with and complements clinical treatment and support offered to young people and families accessing CAMHS services.
- The Peer Workers primary role is to offer peer support, which is the process of giving and receiving non-clinical emotional and practical support to young people and families.
- Carer Peer Workers draw on experience and perspective of witnessing, walking alongside and supporting a child or young person with mental health challenges and recovery. Their role focuses on supporting families/ carers and significant others.
- Consumer Peer Workers draw on first-hand experiences and perspectives of mental health challenges, service use and recovery. Their role focuses on supporting young people.
- Peer Workers use their (peer work) education and training, and draw from their personal lived experiences of mental ill-health and recovery to:
 - Participate as an active member of the MDT within the CAMHS service;
 - Contribute the perspectives and insights of a young person and/or carer/family to care planning and clinical treatment including for example, through participating in individual case reviews and case conferences;
 - Support clinicians in fostering family and young person engagement and participation in care planning, treatment and recovery; and
 - Contribute to continuous service improvement by actively participating (and contributing a lived experience perspective) in planning days and meetings and other service improvement initiatives.
- Peer Workers will not be engaged in ways that may diminish, reduce or replace partnership and consultation with a diverse range of consumer and carer

representatives as outlined in the [CAHS Consumer Involvement in Healthcare Design and Management Policy](#).

Definitions

Peer Worker: In this document, Peer Worker is used as a collective term for staff employed as part of an MDT, including Consumer Peer Workers, Carer Peer Workers and Peer Support Coordinators.

Principles supporting service delivery

The table below, adapted from the [WA Lived Experience \(Peer\) Workforces Framework](#) sets out six key principles that guide peer work in the mental health (and alcohol and other drug) sector.

Principle	Definition and How it looks in practice
Connection	Connection is the cornerstone of peer work, providing a space where relationships can be cultivated and sustained. It's about sharing experiences, building trust and safety through validating experiences, being understanding and non-judgemental. • Share experiences with purpose • Build trust and safety • Validate experience • Create a place where hope and optimism can grow
Authenticity	Authenticity means bringing your true self to the work while knowing your boundaries and limitations. • Have a willingness to work in developing their own identity and support others in developing theirs • Work within boundaries and limitations • Bring honesty and vulnerability to their work
Diversity	Appreciation for diversity of thought and diversity of beliefs, and an understanding that each person's way is the right way for them. • Understand and accept themselves and others as they are • Be welcoming and inclusive • Respect and treat others as individuals
Humanity	Humanity is about recognising the inherent value of all individuals who are shaped by life experiences and are deserving of respect, opportunities, and compassion. • Show compassion • Listen deeply • Be kind and caring
Mutuality	For peer workers, mutuality means bringing the qualities of genuine friendship to peer work rather than a professional to client

	relationship. It's about intentionally developing and maintaining a two-way, equal relationship based on the kinship of common experience. • Support people by 'being with' rather than 'doing for' • Sharing power • See others as equals • Allow for change
Human Rights	Peer workers support people's right to freedom and choice. It's a commitment to fairness, equity and 'a good life.' • Work towards justice and rights for all people • Support inclusion and strong communities • Promote equity and choice • Advocate for all people

Access

For those services with peer workers; all young people and families have the right to request or decline peer support at any point from entry to exit (if available in any given service). However, the ability to offer this service will depend on availability and capacity of peer work staff, noting that at present, not all services have access to a peer worker.

Aboriginal young people and their families will be offered support from both the Aboriginal Mental Health Worker and Peer Worker, where Peer Workers are available within a CAMHS service.

Aboriginal staff employed in designated Lived Experience (Peer) positions can provide peer support to all young people and families, including but not limited to, those that are Aboriginal.

Length of engagement with Peer Support

To promote effective working relationships, it is important to ensure that young people and families have a clear understanding of the length of time the CAMHS Peer Worker is able to offer them peer support. To achieve this, Peer Workers should discuss this as part of their initial and ongoing interactions with the young people and families they support.

Peer support can be provided to young people who have been referred to and/or are active with a CAMHS service, or time limited post-discharge follow-up as described in service work instructions, entry protocols and/or models of care.

The duration and frequency of peer support should be determined in consultation with the MDT, including the Case Manager, Peer Worker, and when needed the Service Manager.

Young people and families have the right to pause or cease involvement with the Peer Worker at any time.

Peer Work in CAMHS - In Scope and Out of Scope

The tables below provide an overview of activities which are in and out of scope for Peer Workers within CAMHS. More specific detail will be included in service-specific entry protocols, work instructions and/or Models of Care (in development). This will include information about how service users may request peer support, if it is available within that particular service/MDT.

Function	Activities
IN SCOPE	
Peer support	• Understand, and purposefully draw upon their own and the collective body of knowledge of Lived Experience for the benefit of the service and service users. • Provide non-clinical support to young people and families on waitlists. • Support young people and families through the orientation process for the service. • Provide peer support, which may include working directly with young people and families, and through groups and/or alongside clinicians. • May attend a home or community visit together with the visiting clinician or an Aboriginal Mental Health Worker in line with the CAMHS Home and Community Visits Procedure. • Provide young people and carers/ families with information and support regarding: ○ the impact of mental health conditions on the entire family ○ how to manage a crisis. • Support young people and families during treatment planning, case review and case conferencing with clinical staff, on request of the service user. • Develop and implement education/workshops for young people and families, including as facilitator or co-facilitator. • System navigation – Provide information on appropriate community services and resources and support young people and families to find and engage with relevant service providers, including those external to CAMHS. • Assist young people and families to provide service feedback (complaints and compliments) and complete surveys i.e. YES and CES surveys.
Clinical support	• Inform clinician after a disclosure or identification of individual patient risk. • Help to bridge the gap between young people and families and the service / treating clinicians e.g. explaining the service, process and terminology through a lived experience perspective. • Active collaboration as part of the MDT, including attending and actively participating in daily huddles/ handover and the

	weekly MDT meeting (Refer to the CAMHS Multidisciplinary Team Clinical Reviews Policy). • Co-facilitate education sessions for young people or families e.g. TCI-F. • Support young people and families to ask questions and share information with the service. • Proactively represent the perspectives of the young person and family, including attending clinical reviews and other appointments on request of the service user. • After development of the care plan, and on request of the service user, may: ○ Assist in the development and review of documentation (e.g. care plans) by providing a lived experience lens. ○ Participate in formulation of treatment, from a lived experience perspective.
Advocacy and representation	• Advocate with, and for, families and young people in relation to services. • Promote the rights of families/ carers and principles of carer participation and recovery. • Represent the lived experience perspective at the service management (operational) level and collaborate as part of the Lived experience (peer) workforce to provide a collective knowledge of young people and family issues in service development. • Assist in the development and review of all service specific information given to young people and families e.g. triage documents, welcome packs. • Interagency liaison/knowledge building – connect with support agencies in the community to enable better interagency relations, referrals and support.
Education and training - staff	• Provide informal orientation to new staff within a service on the Peer Worker role. • Provide education to staff about issues from a lived experience perspective, including presentations at orientation and other training. • Contribute to the development of training for staff, families and young people.
Wellbeing	• Discuss and share methods of self-care and wellbeing strategies with young people and families. • Develop and implement family and/or young people wellbeing initiatives.
Quality, research and evaluation	• Participate in service evaluation, quality improvement and research, including bringing a lived experience lens to research meetings and project working groups specific to their directorate or service area.

OUT OF SCOPE

- Complete administrative tasks that are not related to their role.
- Complete clinical work i.e. assessments, referrals or safety plans.
- Complete security tasks such as escort young people or families/ carers from clinical areas when they are unwilling/reluctant to leave.
- Escort involuntary young people without a clinician.
- Provide 1:1 specials.
- Provide individual client transport or any transport without the appropriate Risk Assessments and Action Plan completed with the Manager.
- Mental Health Act support (this is the role of Mental Health Advocacy Service and/or Health Consumers Council and/or Office of the Chief Psychiatrist).
- Complaints management (this is the role of CAHS Consumer Engagement).
- Peer Workers are not to be involved in the use of coercive or restrictive practices.
- Peer Workers do not attend Mental Health Tribunal Hearings, even if the young person or family/ carer has asked them to attend. The Mental Health Advocacy Service can provide an advocate.

Documentation

As employees, and as active members of the MDT, Peer Workers are required to meet all Department of Health and CAMHS documentation requirements.

All care provided by Peer Workers will be documented in the client/patient's medical record (including PSOLIS and DMR), as per the [CAMHS Medical Records Management Procedure](#).

Privacy and Confidentiality

The specific lived experience details of all staff are confidential, and it is up to them alone to decide how and when to share this personal information. Other staff members are not to investigate or share this information on behalf of other staff.

The [CAHS Information Breach Response Procedure](#) outlines the process which must be followed in response to all suspected information breaches within Child and Adolescent Health Services (CAHS) and provide staff with practical guidance in dealing with a suspected information breach.

As per the [CAHS Confidentiality, Disclosure and Transmission of Health Information Policy](#), *'All staff with access to Health Information as part of their role have a duty to maintain the confidentiality of all information acquired, created and disclosed to them.... any breach of confidentiality may lead to disciplinary action, including the matter being treated as a serious misconduct as per the WA Health Discipline Policy'*.

Reflective Practice

Peer Workers have access to reflective practice as outlined in the CAMHS Lived Experience (Peer) Workforce Reflective Practice Procedure.

References and related external legislation, policies, and guidelines
Accreditation Requirements for Mental Health Services
Mental Health Act 2014 (WA)
Final Report: Ministerial Taskforce into Public Mental Health Services for Infants, Children, and Adolescents (ICA)
WA State Priorities Mental Health, Alcohol and Other Drugs 2020 – 2024
WA Lived Experience (Peer) Workforces Framework
Western Australian Mental Health, Alcohol and Other Drug Services Plan 2015–2025.
National Safety and Quality Health Service Standards
Related CAHS internal policies, procedures and guidelines
CAHS Confidentiality, Disclosure and Transmission of Health Information Policy
CAHS Consumer Involvement in Healthcare Design and Management Policy
CAHS Information Breach Response Procedure
CAHS Workplace Adjustment Guide
CAMHS Home and Community Visits Procedure
CAMHS Medical Records Management Procedure
CAMHS Multidisciplinary Team Clinical Reviews Policy
CAMHS Lived Experience (Peer) Workforce Reflective Practice Procedure

This document can be made available in alternative formats on request.

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